Special Topic: Childhood Sexual Abuse and Adult Development

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CCP-630 - Human Development Across the Lifespan

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April 6th, 2025

The childhood pathway to normal healthy development ends with an adult finding themselves capable of secure attachment, functional employment, and consistent prosocial behaviors. Along this pathway, an arsenal of traumas alter the course. Childhood Sexual Abuse (CSA) is one of many adverse experiences in this arsenal. The impact of CSA is broad, but its overall effect is generally found to be small. CSA is associated with seeing more sexual and physical violence, neglect, lower socioeconomic states of parents, and mental health issues of parents. Childhood sexual abuse is often not a standalone act: it represents one strand of a web, made up of the childhood context that allows CSA to occur, one in which the parent (s) is struggling financially, emotionally, or with substances, or perhaps is absent due to incarceration, death, or child welfare cases calling for separation. It can also occur in a perfectly attuned and attentive family, with all of the resources available to them, but who are part of a community that covers up or hides CSA by minimizing its effects and denying its existence. There is always the possibility that the parent is the perpetrator as well. There is evidence of a pathway for victims of CSA to become perpetrators.

As a society, we must grapple with the question: how do we handle perpetrators who are also victims? What rate of recidivism is worth rehabilitation when the crime is this terrible? What impact can we have on society if we pursue treatment with the hopes of eliminating CSA if we do not know that treatment can be 100% effective? How much should we invest in the treatment of CSA? By looking at how CSA impacts parenting, incarceration and criminal behavior, and mental health outcomes, this paper investigates the downstream impact of CSA and what greater investment in treatment might accomplish.

Victims as Parents

As parents, former victims of CSA may hold a flip side of the coin: the truth of the reality of violence often brings deep reflection on how to avoid it. Victims of CSA are highly aware of the possibility that abuse may occur to their children, and they may be more realistically protective of their children, perhaps overly so. One participant in a mixed methods study said, "*I'm very mindful of who I leave my children with. I suspect everyone. Even close family and my husband as you literally never know until it's too late*

(Lange et al., 2020, p.5)." Of course, taking this notion too far can bring a person to a protective, defensive stance that is isolating or damaging.

Another victim explained that it made her want to have open communication with her children and to establish boundaries. "*I am also very, very open with my children about their bodies, their privacy and their own boundaries. No topic is out of bounds because I strongly believe that open communication is the best form of protection. Abuse thrives in private…(Lange et al., 2020, pg 5)"* Another participant echoed this sentiment: *"It has actually made me more open with my children in terms of making sure they feel able to communicate safely with me. I have not told my children about my own abuse experience but I do educate them on these matters (Lange et al., 2020, p. 8)."*

Other participants found that being the loving parent that can be trusted for their children was a healing experience, or one that also brought envy. On the downside, parents reported not being able to breastfeed or bathe their children because of discomfort with their own or their children's body parts due to CSA. Others reported feeling distant or entering mental health crises because they were triggered by their children turning a certain age that reminded them of themselves.

Incest victims were found to be delayed in their ability to make meaning of their experiences, because of the trauma and confusion of incest (Gul et al., 2020). Incest occurs in a power imbalance, often with a caretaker who is insisting that what they are doing is "love". The damage created by this violation contrasts with this messaging, making it difficult for victims to make sense of the abuse or their abuser. Incest victims also commonly reported that their perpetrators 'became someone else' during the acts of incest, greatly impacting the trust capacity for victims.

Additionally, incest victims reported that they wished they could erase all memory of their perpetrator, especially the happy ones, as it was too confusing and conflicting. It is easy to see how this might impact secure attachment ability, cause trust issues, and

impact future relationships negatively, especially with caregivers or surrounding physical intimacy. CSA through incest has been found to impact attachment by resulting in enmeshed and disorganized attachment (Mcmaugh, 2024).

Path to Incarceration

Plummer and Cossins (2018) found that the cycle of sexual abuse that is commonly thought of bisected by gender, as most victims of CSA are female, but most perpetrators are male. Cossins (2000) examines masculinity through a lens of power and powerlessness as a result of their social relationships with other men. Cossins hypothesizes that the greater a boy's experiences of powerlessness in a context that informs his sexuality, the more likely he will be to seek out sexual experiences with the less powerful, such as children. ''...powerlessness and sexuality become central and defining features of his masculinity, and, hence, his experiences of self-worth.''

Community samples of women who report rape average between 15%- 24% (less in phone interviews and more in-person interviews) while studies involving incarcerated women range from 50%-75% (Malloy et al., 2021, McDaniels-Wilson and Belknap, 2008). Similar discrepancies were reported for CSA, with community reporting well below those for incarcerated women. Female child sex offenders reported up to 80%: more than general population, non-sexual female offenders, and male child sex offenders (Augarde and Rydon-Grange, 2022). Additionally, compared to these groups, female child sex offenders who were victims of CSA reported earlier, more severe and intense childhood sexual abuse over a longer period. These associations highlight the importance of prioritizing treatment for children and adults after rape and CSA, in addition to reducing the incidence of CSA. Additionally, this may be lessened by breaking the cycle through treatment of victims.

The implication regarding temporal precedence of CSA leading to incarceration is causality. The specific cause and effect is theorized to be this: CSA is a trauma that may cause long-term impact on the mental well-being of individuals. Common and easy coping mechanisms such as aggression, substance use or risk-taking may lead to arrest or involvement with the criminal justice system

(Svingen, 2023). PTSD may be an outcome of CSA, and PTSD severity is positively correlated to frequency of arrest (Calhoun et al., 2004) and greater trauma and PTSD is found in forensic/incarcerated populations (Bianchini et al., 2022). Childhood trauma related to sexual violence may cause incarcerated or unincarcerated people interacting with law enforcement to be more reactive to real or perceived threat of violence or attack, which can escalate or exacerbate these interactions (Hepp et al., 2021).

Mental Health Outcomes

While a variety of studies have been conducted looking at the relationship between childhood sexual abuse and mental health in adulthood, review papers show mixed results. A 2019 umbrella review (Hailes et al.) found high quality systematic reviews of the positive association between childhood sexual abuse and a) post traumatic stress disorder b) substance use and c) schizophrenia. The strongest associations reported for psychiatric conditions were between CSA and borderline personality disorder, conversion disorder, anxiety and depression, although the quality of these associations was found by these reviewers to be low to moderate.

A burden of proof study (used to inform policy using a five star rating system) found that alcohol use disorders and self-harm warranted a 3-star rating in their association with CSA. Two-star ratings were given to major depressive disorder, anxiety and asthma (Spencer et al., 2023). Both of the above mentioned review papers highlight the need for more research and evidence-based practices for treatment and prevention of CSA.

Conclusion

CSA has an impact that tends to be cumulative, additive, and synergistic. Victims may experience different outcomes based on severity or frequency of CSA, gender of perpetrator, age of abuse, and additional concurrent childhood trauma, including a lack of support system to address the impact of CSA. Damaged trust and interpersonal skill development may impact relationships long into the future, possibly resulting in isolation and weakened social networks, contrasting the expected successful development of secure attachment, deep and lasting relationships and bonds within a social network, and successful navigation of conflict and repair.

Additionally, victims of CSA are more likely to find themselves as victims of sexual violence as adults, with mediating factors of anger, sensitivity to rejection in relationships, and abuse-related shame (Hébert et al., 2021). A protective factor against revictimization has been found in perceived parental protection (Scoglio et al., 2021).

The impact of CSA is complex, and many routes may lead to many outcomes. The 'dark figure' (Cashmore and Shackel, 2013) represents the many who never disclose (Latiff et al., 2024), leaving a wide gap in knowledge. In spite of all the evidence, CSA is not a set path to mental health illness, troubled parenting, or incarceration. Prevention, treatment and healing are possible, as research continues to inform and refine our understanding of the best pathways forward. Because victims of CSA were often betrayed by a caregiver in a vulnerable time, building trust and relational quality in treatment may be difficult. Dissociation often protects these clients from memories of trauma when these memories are likely to come up or when issues surrounding them are discussed, as in therapy. According to Lawson and Akay-Sullivan (2020), this can negatively impact therapy by contributing to a "failure identity", which keeps clients expecting interventions not to work.

Varese et al. (2021) found positive results for patients with trauma backgrounds by targeting dissociation reduction. Through engagement, CBT of presenting problems, appropriate psychoeducation and normalization of dissociation and other trauma responses, dissociation reduction in the form of distress tolerance, grounding techniques, low arousal and identifying triggers. Importantly, targets selected for intervention were informed by client preference. Strong alliance and repair can benefit clients with a history of CSA, as can helping them distinguish between safe and unsafe people (Lawson and Akay-Sullivan 2020). Because of the link between CSA and dissociative disorders, therapy should include memory/trauma processing only if appropriate and not inducing dissociation. Thus, therapies should be attachment, trauma, and dissociation-informed for victims of CSA.

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